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PRACTICAL GUIDE · EDITION 1.0

Pharmacovigilance Auditing

A practical guide for audit planning, conduct,
and continuous improvement of PV systems.

Pharmacovigilance · Life Sciences · Drug Safety
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00 FOREWORD FROM QUALIVIO

Why this guide exists

Pharmacovigilance auditing sits at the intersection of two disciplines that do not always speak the same language: the precise, evidence based world of drug safety, and the systems thinking discipline of quality assurance. Auditors are expected to understand both, and to translate findings into actions that protect patients.

This guide is written for that audience. It is not a substitute for the legislation, nor a replacement for the Good Pharmacovigilance Practice (GVP) modules published by the European Medicines Agency. Those documents remain the authoritative source. What this guide offers is a working perspective on how to apply them: where to focus, what to look for, and how to make audits genuinely useful rather than merely compliant.

We have written it for a broad audience of PV professionals: auditors building their first programmes, QPPVs who need to understand what good audit oversight looks like, and operational teams who want to know what to expect when an auditor knocks on the door.

Qualivio's view is straightforward. An audit that produces a tidy report but changes nothing has failed. An audit that surfaces an uncomfortable truth, and is followed by genuine corrective action, has done its job. The aim of this guide is to support the second kind.

HOW TO READ THIS GUIDE

Sections 1 to 3 cover the foundations: legal basis, audit types, and how to scope risk based audits.

Sections 4 and 5 walk through the audit lifecycle from planning to follow up.

Section 6 offers focused notes on the highest risk areas of a PV system.

Section 7 closes with practical guidance on health authority inspections.

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01 FOUNDATIONS

The Legal and Quality Basis for PV Audit

Marketing Authorisation Holders in the European Union have been legally required to audit their pharmacovigilance systems since July 2012, when Directive 2010/84/EU and Regulation (EU) No 1235/2010 took effect. The obligation is straightforward to state: an MAH must operate an appropriate pharmacovigilance system, and must verify that system through risk based audit. The detail of how to do that is found in GVP Module IV, supported by Module I on quality systems and Commission Implementing Regulation (EU) No 520/2012 on minimum requirements.

In the United Kingdom, the same obligations now live within the Human Medicines Regulations 2012 as amended, principally regulations 180 and 184. Northern Ireland authorisations remain aligned with EU requirements through CIR Article 13(1) and 17(1), while Great Britain authorisations follow HMR Schedule 12A. The practical outcome is that an MAH operating across both jurisdictions must satisfy two overlapping but not identical regimes.

What counts as an audit

Both GVP Module IV and the ISO 19011 standard converge on the same definition. An audit is a systematic, disciplined, independent and documented process for obtaining evidence and evaluating it objectively against defined criteria. The four adjectives carry the weight of the definition. Each one excludes something.

- Systematic excludes the casual walk through. An audit follows a plan.
- Disciplined excludes the informal opinion. Findings are based on evidence, not impressions.
- Independent excludes the auditor with operational responsibility for the area being reviewed.
- Documented excludes the verbal debrief that leaves no trail. Audit conclusions must survive scrutiny months or years later.

The 2015 update to GVP Module IV made an additional clarification that auditors still occasionally need to defend. Benchmarking, qualification reviews, risk assessment questionnaires and surveys are not audits. They may inform an audit, but they do not substitute for one. The distinction matters because regulators look for genuinely independent evaluation, not self assessment dressed up in audit vocabulary.

What an audit is for

The purpose of PV audit is to provide assurance that the internal pharmacovigilance system, and the related systems that interact with it, are robust and compliant with the applicable standards. Those standards include regulatory requirements, contractual commitments, and the MAH's own procedural documents. The ultimate aim, behind all the procedural language, is the protection of public health.

QUALIVIO PERSPECTIVE

Auditing PV well requires fluency in two registers. The first is the regulatory text: GVP modules, ICH guidelines, national legislation. The second is the operational reality of how a PV system actually works on a Tuesday afternoon when a serious unexpected adverse reaction lands in the inbox at 16:55.

Auditors who can move between the two registers without losing accuracy are rare, and worth investing in.

02 AUDIT UNIVERSE

Audit Types and When to Use Them

A mature PV audit programme will use several different audit types over a multi year cycle. Each type answers a different question, and choosing the wrong type for the question at hand is one of the more common ways an audit programme delivers less value than it should.

Process and system audits

These audits examine a critical PV process across its full lifecycle, or a defined segment of it. The scope can be end to end (for example, every step from case receipt to regulatory submission) or risk focused on the portion where weakness is suspected. Typical subjects include QPPV oversight, aggregate report production, signal management, risk management plan execution, and global case processing including database management.

System audits are the backbone of the programme. They are also the most demanding to scope well, because the auditor must understand how the process is supposed to work before being able to recognise where it has drifted.

Affiliate and local office audits

Affiliate audits review compliance at the national level: how the local operating company implements global procedures, how it manages national reporting requirements, and how it handles local relationships such as patient support programmes or licensing partners. The full population of affiliates forms an audit universe from which selections are made on risk grounds.

A useful test when scoping an affiliate audit is to ask whether local oversight of third parties (vendors, local service providers, distribution partners) falls within scope. If those relationships are not separately audited at the global level, the affiliate audit is usually the natural place to examine them.

Third party and contractual audits

Contractual partners come in several varieties. A co marketing partner with its own marketing authorisation is a different proposition from a contract research organisation processing cases on the MAH's behalf, which is different again from a vendor running a patient support programme. Each requires an audit framed against the contractual obligations as well as the underlying legislation.

The general principle is that legal accountability cannot be delegated, even when activities are. An MAH that outsources case processing remains responsible for the timely reporting of those cases. The audit must therefore examine both the contracted party's performance and the MAH's oversight of that performance.

Verification and follow up audits

After significant findings, an MAH must demonstrate that corrective and preventive actions have been implemented and are effective. Verification can happen at the next routine audit of that area, or as a standalone exercise. The latter is often appropriate when findings were significant enough to feature in the PSMF, since the PSMF entry should only be closed once independent evidence confirms the CAPA has worked.

Due diligence and pre engagement audits

Audits conducted before an acquisition, before signing a new service provider, or before entering a new co marketing arrangement serve a different purpose. They support a commercial decision and identify risks that will need to be managed if the deal proceeds. In the absence of a contractual framework, the audit standards are global legislation, relevant guidance, and the auditing organisation's own policies.

Mock inspections and product audits

A mock inspection mimics the look and feel of a specific regulator's inspection. Because the FDA, MHRA, EMA national competent authorities and others all conduct inspections differently, a mock inspection must be labelled clearly: an MHRA mock inspection prepares the organisation for something quite different from an FDA mock inspection. Generic mock inspections tend to satisfy nobody.

A product audit cuts across multiple processes for a single product, similar in scope to a product related inspection under GVP Module III. These audits are logistically demanding because they require coordination across many functions, but they reveal the kind of product specific weaknesses that process audits can miss.

03 SCOPE

Scoping a PV Audit

Scope is where most audits succeed or fail. A scope that is too broad produces a thin audit that grazes the surface of many areas without going deep on any. A scope that is too narrow can miss the systemic issue sitting just outside the boundary the auditor drew.

Functional areas to consider

PV does not exist in isolation. Drug safety activities depend on, and feed into, a number of other functions. Any of the following may be in scope depending on the audit type and the risk profile of the products involved:

- Drug Safety, including the QPPV office
- Medical Information and customer contact channels
- Product Quality and complaint handling
- Regulatory Affairs
- Medical Affairs and clinical teams
- Commercial, Sales and Marketing
- Market Research, Health Economics and Outcomes Research
- Legal
- Information Technology and computer systems

The auditor's focus is rarely the function in isolation. It is the interaction between that function and the PV system: how spontaneous safety information is captured by a sales representative and routed to drug safety, how a product quality complaint with an adverse event component is reconciled across two databases, how a market research questionnaire is screened for adverse events before the report is finalised.

Critical PV processes

GVP Module I, section I.B.11.3, identifies a set of critical processes that any PV audit programme must cover. They form a useful baseline when constructing the system audit programme, and they should be present, in some form, within affiliate audits as well:

1. Collection, processing, quality control, follow up, coding, evaluation and timely electronic transmission of individual case safety reports from any source.
2. Signal management through continuous safety profile monitoring and benefit risk evaluation.

3. Scheduling, preparation, submission and assessment of aggregate reports including PSURs and DSURs.
4. Meeting PV commitments and responding to competent authority requests with correct and complete information.
5. Interaction between PV and product quality defect systems.
6. Communication of safety concerns between MAHs and competent authorities, including notification of changes to the risk benefit balance.
7. Establishing, assessing and implementing risk management systems, and evaluating the effectiveness of risk minimisation.
8. Communication to patients and healthcare professionals about changes to the risk benefit balance.
9. Keeping product information up to date with current scientific knowledge.
10. Implementation of safety related variations to marketing authorisations according to the urgency required.

These ten activities are not exhaustive, but an audit programme that systematically covers them across a multi year cycle will satisfy most regulatory expectations for breadth.

Two questions that sharpen scope

BEFORE FINALISING SCOPE, ASK

1. What is the worst outcome this audit is designed to detect or prevent? If the answer is fuzzy, the scope is fuzzy.
2. If a regulator inspected this same area tomorrow, where would they look first? That is usually where the audit should look first too.

04 LIFECYCLE

Planning, Conduct and Reporting

Audit lifecycles vary in their stage names but converge on the same logical structure: a long term view that defines the audit universe, an annual plan that selects from it on risk grounds, an operational plan for each individual audit, the audit itself, and the closing communications that turn evidence into action.

Strategic planning: the audit universe

The audit universe is the complete population of auditable units across the PV system: every process, every affiliate, every contracted party, every system. A typical multinational MAH will identify several hundred such units. The strategic plan, covering two to five years, defines how that universe will be sampled.

Risk is the organising principle. GVP Module IV expects the organisation to apply a documented risk assessment process, and to demonstrate that the audit plan flows from it. The conventional formulation is likelihood multiplied by impact, but in PV the impact dimension is asymmetric: an unlikely event that compromises patient safety remains high risk even at low probability.

Useful inputs to the risk assessment include deviation data from the PSMF, prior audit findings, regulatory intelligence about authority focus areas, complexity of the affiliate or partner, and the product portfolio's risk profile. Reviewing PSMF deviations during annual planning is specifically encouraged by GVP Module II.

Tactical planning: the annual plan

The annual plan converts the strategic view into concrete commitments. It is approved by senior management, included in the PSMF, and shared with the business. Sharing is not a weakness. Advance notice allows the business to plan logistically, to prepare materials thoroughly, and in some cases to fix obvious gaps before the audit arrives. An auditor whose primary aim is to find gaps for the inspection report misunderstands the purpose.

Operational planning: individual audit scope

Each audit needs its own scope document and agenda. The scope identifies the boundaries of the process being audited, the samples to be reviewed, the functions to be interviewed, and the criteria against which evidence will be evaluated. Risk based sampling, expected by GVP Module IV, means that not every process step gets equal attention. A PSUR systems audit might focus on a product with an emerging safety concern rather than a stable mature product, on the grounds that any non compliance has more potential to affect patient safety.

The agenda translates scope into time. It usually shows daily start and end times, the timing and focus of each interview, the auditor responsible for each session, and a record of who was interviewed.

Conduct

Audits use three primary evidence sources: document review, interview, and direct observation. Document review provides the paper trail. Interviews test understanding and reveal the difference between procedure and practice. Direct observation, including database demonstrations and facility tours, confirms that what is described in interview actually happens.

Remote and hybrid audits have become permanent fixtures of the landscape. They demand more rigorous preparation, particularly on document logistics, and they tend to expose any weaknesses in the auditor's ability to read non verbal cues. A good remote auditor compensates by asking more open questions and by triangulating across more sources.

Closing and reporting

The closing meeting communicates the auditor's provisional conclusions to the auditees and to management. Whether to share draft ratings at this stage is a matter of organisational policy. Some organisations prefer to wait until the report has been reviewed; others share provisional ratings to avoid surprises later. Either approach can work, but the chosen approach should be consistent and documented.

The audit report is the durable record. It identifies findings, grades each one, and forms the basis for follow up actions. The QPPV and the auditees must both receive the report, per GVP Module IV. Grading definitions can be drawn from GVP Module IV or developed internally; if internal definitions are used, the organisation should check that a finding the regulator would call major is not accidentally classified below the threshold that triggers PSMF inclusion.

CAPA and PSMF update

After the report, the work begins. GVP Module IV expects root cause analysis and a CAPA plan where appropriate. Important findings must be escalated to senior management promptly so that urgent action can follow. The PSMF must be updated to reflect scheduled and completed audits, main findings, and associated CAPAs. Significant findings should only be removed from the PSMF after independent evidence confirms the CAPA has been implemented and is effective.

05 EFFECTIVENESS

From Findings to Sustainable Change

An audit programme is judged ultimately by what changes because of it. A programme that produces dozens of reports per year but cannot point to systemic improvements in compliance, in patient safety outcomes, or in the maturity of the PV system, has become a ceremony rather than a control.

What makes a CAPA effective

CAPAs frequently fail in predictable ways. The most common pattern is a corrective action that addresses the specific instance found by the auditor without addressing the system level cause. Adding a checklist for one process, while the underlying training gap that produced the error remains untouched, is correction without prevention.

SIGNS OF A WEAK CAPA	SIGNS OF A STRONG CAPA
→ Action is identical to the finding, reversed ("will not happen again") → Single root cause identified within an hour of seeing the finding → Owner is the person who made the error, not the system owner → Effectiveness check is the absence of a recurrence within 30 days → Same area generates repeat findings audit after audit	→ Action addresses cause, not symptom, and is independently verifiable → Root cause analysis used a recognised method and considered multiple factors → Owner has authority to change the system, not just the local process → Effectiveness measured by a leading indicator over a meaningful period → Trends across audits show fewer findings in mature areas over time

Escalation

Important findings need to reach senior management quickly. The QPPV in particular must be informed of anything that affects the integrity of the PV system or the safety profile of a product. Escalation routes should be defined in the QA QMS and tested periodically. An escalation route that has never been used in practice often turns out to be slower or less reliable than expected when it is needed in earnest.

Closing the loop with the PSMF

GVP Module II requires that the PSMF includes a list of scheduled and completed audits and a note on the main findings and CAPAs. These sections must be kept current within timeframes defined in company procedures. The discipline of maintaining them correctly forces a useful feedback loop:

anything significant enough to be in the PSMF is significant enough to be tracked through to effective resolution.

A USEFUL TEST

If the PSMF said today that an audit finding from eighteen months ago has been resolved with effective CAPAs, would the evidence behind that statement survive a regulator asking to see it?

If the answer is anything other than a confident yes, the CAPA has not yet been closed.

06 FOCUS AREAS

Focus Areas Across the PV System

The remainder of this guide offers focused notes on areas that recur frequently in PV audit programmes and in regulatory inspections. The notes are deliberately practical rather than encyclopedic. For each area, the aim is to give an auditor a starting point: the questions worth asking, the documents worth requesting, and the patterns that tend to indicate deeper issues.

A general principle applies throughout. The basic requirements for any process to be supported by a quality management system, with procedural documentation, trained personnel, controlled records, and compliance monitoring, are common to every area. Rather than repeating them in each section, they are addressed once in 6.1, and the remaining sections concentrate on what is distinctive to the area in question.

6.1 Quality Management System

The PV quality management system is the foundation on which every other process rests. EU legislation introduced the requirement for an MAH to maintain a quality system for PV activities through Directive 2010/84/EU, and Commission Implementing Regulation (EU) No 520/2012 defines the minimum requirements. Equivalent obligations exist in the United States under 21 CFR 314.80 and 310.305, and in most major jurisdictions.

Auditing the QMS can be done as a standalone exercise or as a foundational element within any other system audit. The questions below should be considered applicable everywhere.

Procedural documents

- Is there a current index or register of PV procedures, and is it accurate?
- Are documents version controlled, with a clear review cycle?
- Are outdated versions retained but inaccessible to staff who should use current versions?
- Do the procedures cover the critical PV processes identified in GVP Module I?
- Is there a process for incorporating regulatory changes into procedures, and a defined timeline for doing so?
- Is there a business continuity plan that has been tested, including for public health emergencies?

Training

- Have staff with PV responsibilities received initial and ongoing training, scaled to their role?

- Have non PV functions that handle safety information received PV awareness training?
- Have vendors and contractors been trained on the PV expectations relevant to their tasks?
- Is training completion tracked, and is there an escalation route for non compliance?
- Does training comprehension get tested, or is completion treated as proof of competence?

Records and data integrity

- Are records accurate, legible, contemporaneous, original and attributable (the ALCOA principles)?
- Are records retained for the duration of the marketing authorisation plus at least ten years in the EU?
- Are PSMF documents retained for at least five years after the PV system terminates?
- Is access to records, electronic and paper, restricted on a need to know basis?
- Are e signatures, where used, compliant with the applicable regulatory requirements?
- Is the organisation able to demonstrate compliance with applicable data privacy law, including GDPR where it applies?

6.2 The QPPV and the PSMF

The Qualified Person for Pharmacovigilance and the Pharmacovigilance System Master File are the two regulatory artefacts through which an MAH demonstrates that someone is accountable for the PV system and that the system itself is documented. Auditors examine both because each acts as a check on the other: the PSMF describes what the QPPV should be overseeing, and the QPPV's actual practice reveals whether the PSMF reflects reality.

The QPPV

The QPPV must reside in the EU, be appropriately qualified, and be permanently and continuously at the MAH's disposal. The role's responsibilities are defined in GVP Module I. Auditors should examine:

- Notification of the QPPV's name and contact details to the relevant competent authorities and the EMA.
- The QPPV's job description, qualifications, and access to a medically qualified person if the QPPV is not themselves medically qualified.
- Mechanisms for continuous availability, including out of hours arrangements (which the auditor may choose to test discreetly).
- The QPPV's knowledge of and oversight over the global PV system, including SOPs, contractual arrangements, database operations, expedited reporting, aggregate reports and training.
- The QPPV's access to safety information, compliance metrics, audit outputs and risk management plans.
- How handover is documented when the deputy QPPV covers a planned absence.
- The QPPV's interaction with national PV contact persons in member states that require them.

National PV contact person requirements vary considerably outside the EU and post Brexit in the UK. Audit scope should reflect the specific jurisdictions of the products in question.

The PSMF

The PSMF is defined in Directive 2001/83/EC Article 1(28e) as a detailed description of the MAH's PV system. GVP Module II sets out the required content and format. Audit attention is usually best directed to the process of maintenance rather than to text editing of the document itself:

- Is the PSMF maintained in compliance with the required format, with body text and annexes?
- Are contributors identified for each section and annex, with backups?
- Is data provided by contributors quality checked before publication?
- Is the change control process adequate, including a logbook that reflects substantive changes?

- Is the location of the PSMF correctly reflected in the Article 57 database, with changes notified within 30 days?
- Can the MAH provide the PSMF to a competent authority within seven days on request?
- Where management of the PSMF has been outsourced, is the arrangement contractually defined and overseen?

Member states and ex EU jurisdictions, including the UK, may require their own national PSMF or equivalent document. Decide early whether these are within audit scope, since each adds materially to the workload.

6.3 Case Processing and ICSR Reporting

Individual case safety report processing is the part of the PV system that produces the most visible compliance metrics. Late reports show up in regulator inspections quickly, and they are easy to count. That visibility makes case processing a frequent audit subject, but it can also distract from the deeper questions about case quality.

Processing

- How is a valid case defined and what happens when one or more of the four criteria are missing?
- Is the date of receipt correctly determined for cases originating internally and externally, and is it consistent for paper and electronic intake?
- Are spontaneous, solicited and clinical trial cases handled according to their distinct conventions?
- Is medical review documented for all cases, not only serious ones?
- Are causality, seriousness and expectedness assessments applied consistently, including against the correct version of the reference safety information?
- How are special situations such as overdose, misuse, off label use, pregnancy exposure and medication errors identified and handled?
- Is duplicate detection robust, particularly where cases enter from multiple channels?
- Is targeted follow up performed for cases that meet RMP criteria, and is the follow up tracked?

Regulatory reporting

ICSRs are submitted to competent authorities under defined timeframes. The mechanism is increasingly electronic via E2B, but paper and email channels persist in some jurisdictions. Audit attention should cover:

- Validation status of the gateway and any database queries used to extract reportable cases.
- Back up arrangements and management of transmission failures.
- Calculation of day zero, which is one of the more common causes of late reporting.
- Handling of follow up information, nullifications, downgrades and case amendments.
- Compliance metrics for serious cases (7 and 15 day reporting) and non serious cases (90 day reporting where applicable).
- Root cause analysis for late reports, and whether preventive actions actually prevent recurrence.
- Sequencing of reporting rule changes against the effective date of the legislation, so that database configuration is in place before submissions begin.

- Where reporting has been delegated to a third party, the contractual terms and the MAH's oversight.

ONE EFFICIENT TEST

Request a listing of all ICSRs reported late over the audit period, then ask for the root cause analysis for each.

Patterns that recur across multiple late reports usually point to a system level issue that previous CAPAs have not actually fixed.

6.4 Signal Management and Risk Management

Signal management and risk management are conceptually distinct but operationally intertwined. Signals, once validated, feed risk management plans. Risk management plans, in turn, drive surveillance commitments that feed back into signal management. An audit that examines one in isolation usually misses something.

Signal management

GVP Module IX defines the signal management lifecycle: detection, validation, confirmation, analysis and prioritisation, assessment, recommendation and communication. Audit attention should consider:

- Planning and scheduling of signal detection activities per product, with intervals appropriate to product lifecycle and exposure.
- Data sources included, ranging from spontaneous reports to clinical trial data, scientific literature, EudraVigilance and other databases as relevant.
- Quantitative thresholds used for disproportionality analysis, and the qualitative methods used in parallel.
- Tracking of potential signals through to closure with documented rationale.
- Timeliness of progression from signal detection through to reference safety information updates.
- Interaction with PSURs, ICSR processing and labelling.
- QPPV involvement in oversight of product safety profiles.

Risk management

Each relevant product should have a risk management plan. The plan defines the safety concerns, the pharmacovigilance activities planned to characterise them, and the risk minimisation measures intended to reduce their impact. Audit attention should consider:

- Whether RMPs exist for all relevant products and reflect current scientific understanding.
- QPPV authority over RMP content.
- Timely update of RMPs when significant new safety information emerges.
- Tracking of commitments within RMPs, including completion against committed timelines.
- Implementation of additional risk minimisation measures, with evidence that they actually reach the intended audience.
- Measurement of effectiveness, going beyond confirmation that materials were sent to confirmation that behaviour or outcomes changed.
- Communication to affiliates and partners of operational RMP requirements that they must execute.

6.5 Aggregate Reports

Aggregate reports give competent authorities a periodic synthesis of safety information. The Periodic Safety Update Report and Periodic Benefit Risk Evaluation Report apply to authorised products under ICH E2C(R2). The Development Safety Update Report covers products under development under ICH E2F. Each type has its own production schedule, its own content requirements, and its own audit considerations.

Process and quality

- Is there a documented procedure for production, review, quality control and submission?
- Does the procedure accommodate legislative changes and local variations such as Russia, Switzerland or the Arab GVP requirements?
- Have authors and reviewers been trained on the procedure and on the underlying guidance?
- Are database queries used to populate reports validated, with evidence that the output reflects the database content?
- Is quality control built into the process, distinct from the audit?

Content and timeliness

- Do reports contain the required elements in the correct format for the relevant authority?
- Are reports submitted to the right recipients within the right timeframes, in the right format?
- Where competent authority assessment reports have raised questions, are those questions addressed in the next cycle?
- For products sharing an active substance across multiple MAHs, how is one consolidated PSUR produced and reviewed?
- Is source documentation retained per GVP Module I?
- Are reports translated as required by individual national authorities?

Consistency across documents

PSURs, DSURs and RMPs for the same product should tell a consistent story. Reviewing them side by side often reveals inconsistencies that no single document review would surface. Clinical trial exposure figures, identified and potential risks, and missing information should align across documents. Auditors who request the current RMP alongside the most recent PSUR will spot misalignment quickly.

6.6 Computer Systems and Data Integrity

PV systems depend on validated computer systems: the global safety database, signal detection platforms, medical information databases, product quality complaint systems, and the workflow tools that connect them. An auditor does not need to be a computer system validation specialist to audit PV computer systems effectively, but does need to understand what the systems are doing and where the risks sit.

Foundations

- Is there an inventory of computer systems supporting PV, with the business processes they support?
- Are systems validated to a level appropriate to their business criticality?
- Is validation documented through the standard lifecycle artefacts: user requirements, functional specifications, user acceptance testing, validation report, release certificate?
- Is change management controlled, with risk assessed before changes are implemented?
- Are MedDRA updates applied across all relevant systems at the same version?
- Are data backups taken regularly, and have they been tested through actual recovery?
- Are disaster recovery plans documented and exercised?

Access and integrity

- Are user accounts managed actively: granted on the basis of role, removed promptly when no longer needed, reviewed periodically?
- Is unblinded clinical trial data protected from users who should not see it?
- Are administrator accounts kept to the minimum needed to operate the system?
- Are audit trails reviewed for critical changes?
- Where products have been divested or acquired, has data migration been planned, documented and verified?

For a deeper review of computer systems compliance, the PIC/S guidance on Good Practices for Computerised Systems in regulated GxP environments remains the most useful reference.

6.7 Third Parties: Partners and Service Providers

PV activities are increasingly conducted across organisational boundaries. Marketing partners share safety obligations for co marketed products. Service providers process cases, write aggregate reports, run literature searches and operate medical information call centres. The MAH that delegates these activities retains every legal responsibility for them.

The contractual frame

Three documents usually define the relationship: a master commercial agreement, a safety data exchange agreement (SDEA) for PV obligations, and a quality or technical agreement for product quality. The SDEA does most of the PV work. Audit attention should consider whether the SDEA:

- Defines roles and responsibilities clearly enough that no activity falls between the parties.
- Specifies products and territories accurately.
- Defines timeframes and methods for ICSR exchange, including reconciliation.
- Allocates responsibility for the global safety database, signal detection, aggregate reports, the QPPV role and the CCDS.
- Grants both parties the right to audit and defines the conditions under which it is exercised.
- Provides for change control as legislation and the relationship evolve.

Oversight in practice

A well drafted SDEA does not guarantee compliance. The auditor should look for evidence that the MAH actively exercises oversight: regular reconciliations that surface and resolve discrepancies, joint governance meetings with documented decisions, training records that confirm vendor personnel are current, and prompt action when issues are identified.

GVP Module IV revision 1 made an explicit point that questionnaires, qualification reviews and surveys are not audits. They have a place in oversight, but they cannot substitute for the independent evaluation that an audit provides. An MAH whose entire third party oversight relies on questionnaire responses is exposed.

WHERE THINGS GO WRONG

The most common third party failure is not a single dramatic breach. It is the slow drift between what the SDEA says and what people actually do as staff change, processes evolve, and the original signatories move on.

Periodic audit is the mechanism that catches the drift before it becomes a finding from a regulator.

07 INSPECTIONS

Preparing for Health Authority Inspections

A health authority inspection examines compliance with national and regional legislation, applicable guidance, and the MAH's own procedures. Being compliant and being able to demonstrate compliance under inspection conditions are related but not identical achievements. The second requires planning, preparation and disciplined execution.

Inspection readiness as an ongoing state

Organisations that perform well in inspections tend not to treat preparation as a discrete project triggered by announcement of an inspection. They treat readiness as an ongoing state, maintained through the routine quality system. The audit programme is part of that maintenance. Self identified findings closed with effective CAPAs are evidence of a working quality system. Findings that the regulator identifies first are evidence that the quality system has missed something.

What inspectors typically examine in PV

Inspection priorities vary by authority and by inspection type, but the recurring areas of focus include the QPPV and PSMF, ICSR reporting compliance, signal management, risk management plan execution, third party oversight, computer systems and data integrity, and the audit programme itself. The audit programme is examined because the legislation requires it, and because it gives inspectors a view of how seriously the organisation takes self assessment.

Preparing the audit function for inspection

Quality assurance personnel can expect to be interviewed and to have audit documentation requested. Useful preparation includes:

- Being able to articulate the risk assessment methodology that underpins the audit universe and the annual plan.
- Having auditor qualifications, training records and continuing professional development readily available.
- Having audit reports organised and retrievable, with a clear understanding of which sections can be shared and which the organisation may decline on the grounds of protecting the integrity of the audit programme.
- Being able to walk through the CAPA process for a sample finding, from identification through to effectiveness verification.
- Knowing the contents of the relevant PSMF audit sections and being able to reconcile them against internal records.

Logistics of inspection management

Most organisations benefit from a documented procedure for inspection management that covers announcement through to closure. Internal training on the procedure should be refreshed periodically rather than left to be reread in haste when an inspection is announced. Mock interviews with likely subject matter experts can be useful, but only if the underlying processes are actually compliant. Coaching cannot make a deficient process appear compliant under sustained questioning.

After the inspection

Inspection findings, like audit findings, become useful only when they drive change. Root cause analysis should be conducted with the same rigour applied to internal findings. Responses should address cause rather than symptom. Sharing of inspection findings between authorities means that a poor outcome with one regulator can trigger interest from others, so the response strategy should consider the regulatory ecosystem rather than the single inspecting authority.

PRINCIPAL REFERENCES

EMA Good Pharmacovigilance Practices, all current modules, available at ema.europa.eu

MHRA guidance on good pharmacovigilance practice (GPvP), available at gov.uk

Commission Implementing Regulation (EU) No 520/2012

ICH E2C(R2), E2D, E2E, E2F

ISO 19011, Guidelines for auditing management systems



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About this guide

This practical guide reflects Qualivio's perspective on pharmacovigilance auditing. It draws on the published regulatory framework, including the GVP modules, EU and UK legislation, ICH guidelines and ISO standards, as well as on the operational experience of working with PV systems in practice.

It is intended as a starting point for auditors, QPPVs and operational PV teams. It is not a substitute for the source documents themselves, nor for the judgement that experienced auditors bring to each individual situation.

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